

02/27/26 12:18:03 Select Medical
Select Physical Therapy
10470 South Progress Way
Suite 104
Parker, CO, USA, 80134
Phone: (303) 617-4700
Fax: (303) 617-4707

Patient: Jennifer Hipskind
Acct #: 035R241624370
DOB: Dec 02, 1994
Clinician: Rachel Severson, PT
Prim Phy: Joel L. Shubert
Phy Phone: (303) 925-4380
Phy Fax: (303) 925-4381
Sec Phy: Not Specified
Inj. Date: 7/13/2025
Surg. Date: Not Specified

18662316344 Select Medical

Page 040

Visit Date: Jan 29, 2026
FSC: HMO/PPO
Payor: AETNA
Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 24
Cxl/Ns: 0

Daily Note

Diagnosis

Spine M54.2 Cervicalgia

General Information

History of Injury: Pt reports neck pain began in June or July, with sudden pain at the base of the skull without MOI. Pain would spread down either side of the neck into the shoulder blades, but did not feel nerve related. Pt describes pain as dull and throbbing. Also felt heat in her neck, needing ice packs to relieve the feeling. Reports no accidents or injuries. Pt sits at a computer for work as a pharmacist and does get monthly massages. Pt saw PCP, C/S x-ray revealed lack of normal curvature, was given steroids and that helped. Pt reports hot feeling has resolved following steroids in August, Robaxin is helping with sleep currently, but still walks up 1-2x in the middle of the night. Most aggravated right after waking, sitting at computer, and the end of the day. Now is using heat pack to relax muscles and feels relief from this. Pt denies illness at the time of first onset. Pt reports daily headaches for the past 2-3 years, were not always daily.

Review of Systems: Reviewed with pt; R shoulder injury from skiing, L shoulder dislocation as child; L ACL surgery in 2017. The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Subjective Examination

Spine

History: Medical:

- **Past/Present:** COVID-19. Headaches. Dizzy or Fainting Spells.

Chief Complaint: Pain: Severity:

- At Best 0/10
- At Worst 3/10
- PROMIS Global Health v1.2: In the past 7 days, how would you rate your pain on average (0-10) 4/10
- PROMIS Pain Interference v1.1 (CAT) - T Score 54.44

History: Medical:

- **Functional Comorbidity Index:** Depression. Upper Gastrointestinal Disease.

ADL / Functional Status: Premorbid Status: No neck pain prior to June/July

History: Medical: Functional Comorbidity Index: Visual Impairment.

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

- Driving/Traveling WNL
- Housework- Light WNL
- Lifting WNL
- Reaching WNL
- Sitting WNL
- Sleeping no longer being woken up due to neck pain; but some stiffness when she does wake up
- Using Computer/Tablet Increased pain sitting at desk at work due to computer work; worsens by mid morning/ end of day; only takes Celebrex or Tylenol when a tension headache occurs

History: Medical: Functional Comorbidity Index: Body Measurements:

- Height (in) 70 Inches
- Weight (lbs) 165 Pounds
- Body Mass Index - Calculated 23.67

Denies: Abnormal Sensation.



The medical history questionnaire including fall risk screening has been completed and signed by the patient, reviewed by the evaluating therapist, and is on file.

Daily Comments: 1/29: She reports a headache on Monday which lasted almost all day. She has been having migraines a lot the last few weeks - she doesn't feel that the migraines are stemming from her cervical tightness. Notes focal suboccipital tightness, but hasn't noted a radiating pain.

ADL / Functional Status: 11/17: The pt. reports over 50% improvement however, continued pain. The pt. reports a couple of times a week, her pain still hurts - laying down most notably she is stiffer. Morning time feels stiffer and more sore, and some days get progressively worse throughout the day. The pt. reports her pain used to be at the base of her skull and now it's into her cervical and upper thoracic spine - stiffness and tenderness present. At her C7, the pt. reports the skin seemed to contract a rash - like eczema and then it peeled off; used to be sharp and stabbing pain. The pt. reports this sharp pain at C7 will continue to come back but less frequently.

Patient Reported Outcomes

PRO Measure	Baseline Score	Latest Score
PROMIS Global Health v1.2	2025-10-11	
Global Physical Health (T-score)	44.9	
Global Mental Health (T-score)	38.8	
PROMIS Pain Interference v1.1 (CAT)	2025-10-11	
Pain Interference (T-score)	54.44	
PROMIS Physical Function v2.0 (CAT)	2025-10-11	
Physical Function (T-score)	54.71	

Objective Examination

Spine

Observations: Cardiovascular System: Pre-Treatment:

- Pulse
- Blood Pressure: Systolic
- Blood Pressure: Diastolic

76 Beats/Minute
110 mm/Hg
83 mm/Hg

Palpation: Tightness b/l suboccipitals, cervical paraspinals, UT R>L, but no pain

Range of Motion: Spine: Pre-Treatment: Active Cervical (%):

- Extension(no crunching like before)
- Flexion
- Rotation Left(tightness)
- Rotation Right(Minor tightness noted in R UT)
- Side Bending Left(tightness/ stiffness)
- Side Bending Right(Stretching feeling)

100%
100%
95%
100%
80%
90%

Muscle Testing: Upper Extremity MMT:

- Shoulder Abduction:
- Shoulder Flexion:
- Shoulder External Rotation:
- Shoulder Internal Rotation:

Left	Right
5/5	5/5
+4/5	-5/5
+4/5	+4/5
5/5	5/5

Joint Integrity/Mobility: Cervical:

- Central P-A

Left	Right
Hypermobile	Normal

Special Tests: Spine: Cervical:

- Spurling's Test

Left	Right
Neg	Neg

Patient:
Acct #:
DOB:Jennifer Hipskind
035R241624370
Dec 02, 1994

Visit Date: Jan 29, 2026

- Upper Limb Tension Test

Treatments**Spine****Exercise Activities:** Aerobic Conditioning:

- Upper Body Ergometer(Initiate next visit)

Manual Interventions: Soft Tissue- Upper Quarter:

- Cervicothoracic PVM(Initiate next visit)
- Suboccipitals

Pt./Family Education:

- Patient Education 1

Exercise Activities: Isotonics(U. Quarter):

- Isotonic Activity 1

- Upright Rows.
- Isotonic Activity 2

- Isotonic Activity 3
- Prone Horizontal Abduction.
- Overhead Press.
- Isotonic Activity 4
- Supine Punch
- Isotonic Activity 5

Exercise Activities:Range of Motion:

- Rotation

Manual Interventions: Joint Segments: Vertebral:

- Cervical Spine

Exercise Activities: Flexibility(U. Quarter):

- Levator Scapulae Musculature
- Upper Trapezius Muscle
- Flexibility Activity 1
- Flexibility Activity 2
- Flexibility Activity 3

Exercise Activities:Tubing/Bands(U. Quarter):

- Flexion, Shoulder
- Horizontal Abduction- Standing
- Tubing/Bands 1
- TRX Rows

- D2 Flexion/Extension.
- Tubing/Bands 2

Neg

Neg

Did Not Perform: Initiate next visit

Did Not Perform: Initiate next visit

Tx Depth: Moderate, Technique: Sustained Pressure, Additional Detail: suboccipital release, Time Elapsed: 7 Minutes, Charge As: Manual Therapy Techniques, Performed by: Rachel Severson, PT, 7 Minutes.

Did Not Perform:

Additional Detail: Quadruped regular chin tuck, Time Elapsed: 6 Minutes, Repetitions: 10, Sets: 3, Description: chin tuck +rotation +head lift, Charge As: Neuromuscular Reeducation, Performed by: Rachel Severson, PT, 6 Minutes.

Did Not Perform:

Time Elapsed: 7 Minutes, Repetitions: 10, Sets: 2, Description: TRX flies, Charge As: Therapeutic Activities, Performed by: Rachel Severson, PT, 7 Minutes.

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Repetitions: 10, Sets: 3, Description: Pilates ring drivers, Time Elapsed: 7 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Rachel Severson, PT, 7 Minutes.

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Time Elapsed: 8 Minutes, Repetitions: 10, Sets: 3, Technique: Active Assistive, Description: FR Series (Swimmers, Angels, Stretch), Charge As: Therapeutic Exercise, Performed by: Rachel Severson, PT, 8 Minutes.

Did Not Perform:

Did Not Perform:

Did Not Perform:

Time Elapsed: 7 Minutes, Repetitions: 10, Sets: 3, Description: TRX high row with chin tuck, Charge As: Therapeutic Exercise, Performed by: Rachel Severson, PT, 7 Minutes.

Did Not Perform:

Repetitions: 10, Sets: 3, Tubing/Band Color: RTB, Resistance:

Concentric/Eccentric, Description: scapular pulses, Time Elapsed: 4 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Rachel Severson, PT, 4 Minutes.

Modalities:

- Moist Hot Pack

Exercise Activities: Nerve Glides:

- Radial Nerve.

Exercise Activities: Stabilization Training: Prone Position:

- Retraction/Extension

Did Not Perform:

Did Not Perform:

Repetitions: 10 x 5 sec hold, Technique: Active, Body Position: prone on table, Additional Detail: prone scap squeezes with chin tuck, ROM Performed: Available, Time Elapsed: 4 Minutes, Charge As: Therapeutic Exercise, Performed by: Rachel Severson, PT, 4 Minutes.

Therapy Session Time

- Therapy Session Start Time
- Therapy Session Stop Time
- Total Therapy Session Time

01/29/2026 10:32 AM

01/29/2026 11:22 AM

50 Minutes

Documented Procedural Code Summary:

Description

Code

Units

Minutes

- Manual Therapy Techniques
- Neuromuscular Reeducation
- Therapeutic Activities
- Therapeutic Procedure

97140

1.00

7

97112

2.00

17

97530

1.00

7

97110

2.00

19

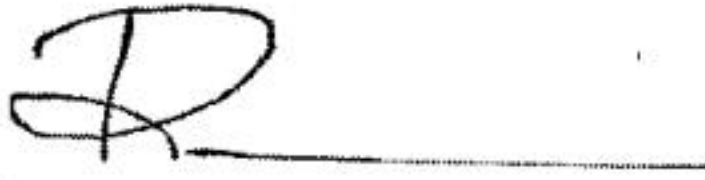
Assessment

Patient tolerated advanced exercise prescription today. HEP updated and patient demonstrated understanding. She benefitted from manual therapy as noted by improved pain and increased ROM for better activity tolerance at the end of session today to target focal tightness at suboccipitals.

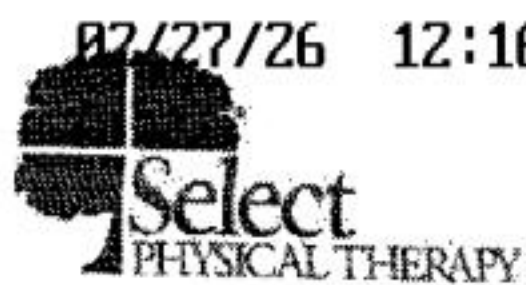
Plan

Spine

Daily Plan: Continue w/ Current Rehabilitation Program.



Rachel . Severson, PT, PT(CO Lic: PTL.0019542)
Signed on Jan 29, 2026 13:03:51



02/27/26 12:16:01 Select Medical
Select Physical Therapy
10470 South Progress Way
Suite 104
Parker, CO, USA, 80134
Phone: (303) 617-4700
Fax: (303) 617-4707

Patient: Jennifer Hipskind
Acct #: 035R241624370
DOB: Dec 02, 1994
Clinician: Rachel Severson, PT
Prim Phy: Joel L. Shubert
Phy Phone: (303) 925-4380
Phy Fax: (303) 925-4381
Sec Phy: Not Specified
Inj. Date: 7/13/2025
Surg. Date: Not Specified

18662316344 Select Medical

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Visit Date: Feb 02, 2026
FSC: HMO/PPO
Payor: AETNA
Insured: Not Specified
Employer: Not Specified
Case Mgr: Not Specified
Visits: 25
Cxl/Ns: 0

Daily Note

Diagnosis

Spine M54.2 Cervicalgia

General Information

History of Injury: Pt reports neck pain began in June or July, with sudden pain at the base of the skull without MOI. Pain would spread down either side of the neck into the shoulder blades, but did not feel nerve related. Pt describes pain as dull and throbbing. Also felt heat in her neck, needing ice packs to relieve the feeling. Reports no accidents or injuries. Pt sits at a computer for work as a pharmacist and does get monthly massages. Pt saw PCP, C/S x-ray revealed lack of normal curvature, was given steroids and that helped. Pt reports hot feeling has resolved following steroids in August, Robaxin is helping with sleep currently, but still walks up 1-2x in the middle of the night. Most aggravated right after waking, sitting at computer, and the end of the day. Now is using heat pack to relax muscles and feels relief from this. Pt denies illness at the time of first onset. Pt reports daily headaches for the past 2-3 years, were not always daily.

Review of Systems: Reviewed with pt; R shoulder injury from skiing, L shoulder dislocation as child; L ACL surgery in 2017. The medical history questionnaire indicates a history of pulmonary disease. Assessment and documentation of preservice respiratory rate and blood oxygen saturation are recommended.

Subjective Examination

Spine

History: Medical:

- **Past/Present:** COVID-19. Headaches. Dizzy or Fainting Spells.

Chief Complaint: Pain: Severity:

- At Best 0/10
- At Worst 3/10
- PROMIS Global Health v1.2: In the past 7 days, how would you rate your pain on average (0-10) 4/10
- PROMIS Pain Interference v1.1 (CAT) - T Score 54.44

History: Medical:

- **Functional Comorbidity Index:** Depression, Upper Gastrointestinal Disease.

ADL / Functional Status: Premorbid Status: No neck pain prior to June/July

History: Medical: Functional Comorbidity Index: Visual Impairment.

ADL / Functional Status: Current Status: ADL/ IADL: Activities:

- Driving/Traveling WNL
- Housework- Light WNL
- Lifting WNL
- Reaching WNL
- Sitting WNL
- Sleeping no longer being woken up due to neck pain; but some stiffness when she does wake up
- Using Computer/Tablet Increased pain sitting at desk at work due to computer work; worsens by mid morning/ end of day; only takes Celebrex or Tylenol when a tension headache occurs

History: Medical: Functional Comorbidity Index: Body Measurements:

- Height (in) 70 Inches
- Weight (lbs) 165 Pounds
- Body Mass Index - Calculated 23.67

Denies: Abnormal Sensation.

The medical history questionnaire including fall risk screening has been completed and signed by the patient, reviewed by the evaluating therapist, and is on file.

Daily Comments: 1/29: She reports a headache on Monday which lasted almost all day. She has been having migraines a lot the last few weeks - she doesn't feel that the migraines are stemming from her cervical tightness. Notes focal suboccipital tightness, but hasn't noted a radiating pain.

2/2: patient had an MRI performed - uploaded document to file. Findings: "minimal cervical spondylosis without disc herniation, canal or foraminal stenosis. There is facet arthropathy on the L at C5-6 related to developmental hypoplasia of the C6 superior articular facet"

- reports last headache was a minor headache on Wednesday with lasted 3-4 hours. No headaches since/over the weekend.

ADL / Functional Status: 11/17: The pt. reports over 50% improvement however, continued pain. The pt. reports a couple of times a week, her pain still hurts - laying down most notably she is stiffer. Morning time feels stiffer and more sore, and some days get progressively worse throughout the day. The pt. reports her pain used to be at the base of her skull and now it's into her cervical and upper thoracic spine - stiffness and tenderness present. At her C7, the pt. reports the skin seemed to contract a rash - like eczema and then it peeled off; used to be sharp and stabbing pain. The pt. reports this sharp pain at C7 will continue to come back but less frequently.

Patient Reported Outcomes

PRO Measure	Baseline Score	Latest Score
PROMIS Global Health v1.2	2025-10-11	
Global Physical Health (T-score)	44.9	
Global Mental Health (T-score)	38.8	
PROMIS Pain Interference v1.1 (CAT)	2025-10-11	
Pain Interference (T-score)	54.44	
PROMIS Physical Function v2.0 (CAT)	2025-10-11	
Physical Function (T-score)	54.71	

Objective Examination

Spine

Observations: Cardiovascular System: Pre-Treatment:

- Pulse
- Blood Pressure: Systolic
- Blood Pressure: Diastolic

76 Beats/Minute
110 mm/Hg
83 mm/Hg

Palpation: Tightness b/l suboccipitals, cervical paraspinals, UT R>L, but no pain

Range of Motion: Spine: Pre-Treatment: Active Cervical (%):

- Extension(no crunching like before)
- Flexion
- Rotation Left(tightness)
- Rotation Right(Minor tightness noted in R UT)
- Side Bending Left(tightness/ stiffness)
- Side Bending Right(Stretching feeling)

100%
100%
95%
100%
80%
90%

Muscle Testing: Upper Extremity MMT:

- Shoulder Abduction:
- Shoulder Flexion:
- Shoulder External Rotation:
- Shoulder Internal Rotation:

Left	Right
5/5	5/5
+4/5	-5/5
+4/5	+4/5
5/5	5/5

Joint Integrity/Mobility: Cervical:

Left	Right
------	-------

Patient:
Acct #:
DOB:Jennifer Hipskind
035R241624370
Dec 02, 1994

Visit Date: Feb 02, 2026

- Central P-A

Special Tests: Spine: Cervical:

- Spurling's Test
- Upper Limb Tension Test

Treatments**Spine****Exercise Activities:** Aerobic Conditioning:

- Upper Body Ergometer(Initiate next visit)

Manual Interventions: Soft Tissue- Upper Quarter:

- Cervicothoracic PVM(Initiate next visit)
- Suboccipitals

Pt./Family Education:

- Patient Education 1

Exercise Activities: Isotonics(U. Quarter):

- Isotonic Activity 1
- Upright Rows.
- Isotonic Activity 2

- Isotonic Activity 3

- Prone Horizontal Abduction.
- Overhead Press.
- Isotonic Activity 4
- Supine Punch
- Isotonic Activity 5

Exercise Activities:Range of Motion:

- Rotation

Manual Interventions: Joint Segments: Vertebral:

- Cervical Spine

Exercise Activities: Flexibility(U. Quarter):

- Levator Scapulae Musculature
- Upper Trapezius Muscle
- Flexibility Activity 1
- Flexibility Activity 2
- Flexibility Activity 3

Exercise Activities:Tubing/Bands(U. Quarter):

- Flexion, Shoulder
- Horizontal Abduction- Standing
- Tubing/Bands 1
- TRX Rows

- D2 Flexion/Extension.

Hypermobile

Normal

LeftNeg
Neg**Right**Neg
Neg

Time Elapsed: 5 Minutes, Direction: forward & backward, Speed (rpm): Self-Paced, Charge As: Therapeutic Activities, Performed by: Rachel Severson, PT, 5 Minutes.

Did Not Perform: Initiate next visit

Tx Depth: Moderate, Technique: Sustained Pressure, Additional Detail: suboccipital release, Time Elapsed: 7 Minutes, Charge As: Manual Therapy Techniques, Performed by: Rachel Severson, PT, 7 Minutes.

Did Not Perform:

Did Not Perform:

Did Not Perform:

Time Elapsed: 7 Minutes, Repetitions: 10, Sets: 2, Description: TRX flys, Charge As: Therapeutic Activities, Performed by: Rachel Severson, PT, 7 Minutes.

Time Elapsed: 6 Minutes, Repetitions: 10, Sets: 3, Description: table top core marches, Charge As: Therapeutic Activities, Performed by: Rachel Severson, PT, 6 Minutes.

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Time Elapsed: 2 Minutes, Grade: III, Body Position: Supine, Additional Detail: C7 central PA, Pressure: Oscillatory, Technique 1: Unilateral P-A, Charge As: Manual Therapy Techniques, Performed by: Rachel Severson, PT, 2 Minutes.

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Did Not Perform:

Time Elapsed: 7 Minutes, Repetitions: 10, Sets: 3, Description: TRX high row with chin tuck, Charge As: Therapeutic Exercise, Performed by: Rachel Severson, PT, 7 Minutes.

Did Not Perform:

• Tubing/Bands 2

Modalities:

• Moist Hot Pack

Exercise Activities: Nerve Glides:

• Radial Nerve.

Exercise Activities: Stabilization Training: Prone Position:

• Retraction/Extension

Therapy Session Time

- Therapy Session Start Time
- Therapy Session Stop Time
- Total Therapy Session Time

Documented Procedural Code Summary:

Description

- Manual Therapy Techniques
- Neuromuscular Reeducation
- Therapeutic Activities
- Therapeutic Procedure

Code

97140
97112
97530
97110

Units

1.00
1.00
2.00
1.00

Minutes

9
4
18
11

Assessment

Patient tolerated exercise prescription today - noted more fatigue today requiring more rest breaks. She benefitted from manual therapy as noted by improved pain and improved ROM which aided post activity to decrease tension. Skilled PT continues to be necessary to strengthen surrounding cervical musculature to provide better stability

Plan

Spine

Daily Plan: Continue w/ Current Rehabilitation Program.



Rachel . Severson, PT, PT(CO Lic: PTL.0019542)
Signed on Feb 03, 2026 08:21:44

Repetitions: 10, Sets: 3, Tubing/Band Color: RTB, Resistance: Concentric/Eccentric, Description: scapular pulses, Time Elapsed: 4 Minutes, Charge As: Neuromuscular Reeducation, Performed by: Rachel Severson, PT, 4 Minutes.

Did Not Perform:

Did Not Perform:

Repetitions: 10 x 5 sec hold, Technique: Active, Body Position: prone on table, Additional Detail: prone scap squeezes with chin tuck, ROM Performed: Available, Time Elapsed: 4 Minutes, Charge As: Therapeutic Exercise, Performed by: Rachel Severson, PT, 4 Minutes.

02/02/2026 04:02 PM

02/02/2026 04:45 PM

43 Minutes